

Intravesical chemotherapy for bladder cancer

Chemotherapy uses anti-cancer (cytotoxic) drugs to destroy cancer cells. Intravesical chemotherapy is when the treatment is given directly into the bladder.

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What is intravesical chemotherapy?

Chemotherapy uses anti-cancer (cytotoxic) drugs to destroy cancer cells.

Chemotherapy can be given directly into [the bladder](#) for non-muscle-invasive [bladder cancer](#). This is called intravesical chemotherapy.

When is intravesical chemotherapy used?

You usually have intravesical chemotherapy after a [transurethral resection of a bladder tumour \(TURBT\)](#). The chemotherapy reduces the risk of the cancer coming back in the bladder lining.

If you have a [low risk](#) of the cancer coming back, you usually do not need chemotherapy after this.

If you have an intermediate risk of the cancer coming back, you may have more intravesical chemotherapy. This is usually given once a week for 6 weeks.

After this, you may be offered more intravesical chemotherapy. These are called maintenance doses.

Having intravesical chemotherapy

You have chemotherapy into the bladder through a tube called a catheter. This tube goes through the urethra and into the bladder.

The chemotherapy drug used most is mitomycin-C. Sometimes, other chemotherapy drugs may be used, such as gemcitabine or epirubicin.

After surgery

After surgery to remove a bladder tumour, you will usually have a catheter draining urine (pee) from your bladder into a collecting bag. Your nurse will give the chemotherapy through this tube.

This may happen when you are still in the operating theatre or recovery room. Or you may have it when you go back to the ward.

Your doctor may decide to delay the chemotherapy if either:

- you have a lot of blood in your urine (pee)
- there has been any injury to your bladder during surgery.

As an outpatient

You have intravesical chemotherapy in the hospital outpatient department. Your nurse or doctor will give you advice on preparing for your chemotherapy. For example, they will ask you to limit the amount of fluids you drink directly before chemotherapy. This can:

- make it easier for you to keep the chemotherapy in your bladder for the required time – this stops you needing to pass urine (pee) too early
- increase the concentration of the chemotherapy drug in your bladder.

Tell your doctor if you take any medications. People who take water tablets should take them after the treatment. These are called diuretics.

When you arrive for your appointment you will be asked for a sample of urine. Your nurse will check the sample for infection. You will not be given chemotherapy if you are feeling unwell or have a urine infection.

During treatment

To have the chemotherapy a nurse passes a tube called a catheter through the urethra and into the bladder. They then slowly put the chemotherapy through the catheter directly into the bladder.

The nurse usually then takes the catheter out and asks you not to pass urine for about an hour. This gives the chemotherapy time to work. You can get up and walk around while waiting for the treatment to finish. Your bladder may feel full and a bit uncomfortable, but you can go to the toilet as soon as the treatment is finished.

Sometimes, the nurse leaves the catheter in and clamps it to keep the chemotherapy in your bladder. When the treatment is over, the nurse removes the clamp and the chemotherapy drains into a urine bag. Your nurse then takes the catheter out.

After treatment

Your nurse may ask you to take certain precautions after treatment. This can include things like washing your hands and the skin around your genital area with soap and water after you pass urine. These precautions can be different depending on your hospital. You should follow the advice given by your nurse.

For about 2 days after each treatment, you will usually be asked to drink at least 2 to 3 litres (3½ to 5 pints) of fluid a day. This helps to flush the drug out of your bladder.

Sex and contraception

You are usually advised not to have sex for at least 48 hours after the treatment as it may be uncomfortable.

Small amounts of chemotherapy can get into your body fluids. This includes fluid made in the vagina and the fluid that contains sperm. To protect any partners, your cancer doctor may advise that for a few days after this treatment you use:

- a condom for vaginal or anal sex
- a condom or a latex barrier such as a dental dam for oral sex.

Your cancer doctor or specialist nurse can give you more information about this.

Your doctor will tell you not to [become pregnant](#) or [make someone pregnant](#) while having chemotherapy to treat bladder cancer. This is because the drugs may harm a developing baby. You should use effective contraception during your treatment. Your doctor or specialist nurse can give you more information about this.

We have more information about how chemotherapy may affect your [sex life](#).

About our information

References



Below is a sample of the sources used in our bladder cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Mottet N, Bellmunt J, Briers E, et al. Non-muscle-invasive bladder cancer (TaT1 and CIS). European Association of Urology (Internet), 2021. Available from uroweb.org/guideline/non-muscle-invasive-bladder-cancer (accessed September 2021).

Witjes JA, Bruins HM, Cathomas R, et al. Muscle-invasive and metastatic bladder cancer. European Association of Urology (Internet), 2021, Available from uroweb.org/guideline/bladder-cancer-muscle-invasive-and-metastatic (accessed September 2021).

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Dr Ursula McGovern](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

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ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help